

Business Audit Report

Operational health check, value leakage assessment and transformation roadmap

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GROWTH IS NOT THE PROBLEM. THE OPERATING SYSTEM IS.

Brightpath has strong patient demand, five-star reviews and a credible clinical proposition. The central audit finding is that the business has outgrown the manual tools supporting it. The report therefore focuses on the operating backbone required before a sixth and seventh location are added.

SOURCE BASIS

This report is based on the Brightpath Health Clinic case brief and follows the structure of the supplied business-audit template while using original wording, layout and visual treatment. Quantified loss estimates are treated as planning estimates rather than audited financial figures.

Executive summary

Brightpath is a growing private healthcare business with approximately £2 million in annual revenue, five West Midlands clinics and a 25-person team. Its customer proposition is healthy; its internal operating model is the constraint. The case brief points to five connected gaps: fragmented scheduling, disconnected patient records, inconsistent marketing, poor cross-site coordination and an absence of live management information.

5 CLINICS	25 EMPLOYEES	~£2M ANNUAL REVENUE
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Audit headline. The supplied audit model estimates roughly **£112,000 per year** of operational leakage across the five problem areas, equivalent to about 5.6% of the stated turnover. The largest single opportunity is scheduling, estimated at about £40,000 per year.

What the case tells us

BUSINESS MODEL B2C private healthcare: physiotherapy, nutrition coaching, mental-health therapy and wellness programmes.	FOOTPRINT Birmingham, Wolverhampton, Coventry, Solihull and Walsall; expansion to two further cities is being considered.
PEOPLE 25 staff across clinicians and reception, supported by one office manager and one marketing freelancer.	FOUNDATION Founded in 2019 by Dr. Amara Okonkwo, a GP turned wellness entrepreneur.
WHAT IS WORKING • Five-star reviews and strong patient satisfaction • Broad service range supports cross-selling • Founder has strong clinical credibility • Word-of-mouth reduces dependence on paid acquisition	WHERE THE ENGINE STRUGGLES • Paper diaries and disconnected calendars • No central patient-data source • One freelancer carries the marketing system • Limited visibility of bookings, retention and revenue

The five friction points

Each issue is assessed as a workflow problem: what happens today, what that creates downstream, the planning-level cost and the practical control that would remove the bottleneck.

01 Manual scheduling	MEDIUM
OBSERVATION Three clinics still rely on paper diaries while the other two use a shared Google Calendar that is not consistently maintained. This creates recurring double-bookings and can send patients to a clinic when the clinician has moved elsewhere.	DOWNSTREAM EFFECT Lost appointments and idle capacity lead to rebooking work, frustration and avoidable churn. Because scheduling touches every clinic, the weakness propagates into capacity management and patient experience.
ESTIMATED LEAKAGE ≈ £40,000 / year £3,000/month of no-shows and rebookings × 12 = £36,000, plus roughly £4,000 in reception coordination time.	CONTROL TO INTRODUCE Move to a single cloud scheduling platform covering all five clinics, with automatic confirmations/reminders and group-wide availability.
02 Patient records are fragmented	HIGH
OBSERVATION Each site keeps its own physical patient files. A patient moving from one service or clinic to another effectively starts again, and clinicians repeat information already collected elsewhere.	DOWNSTREAM EFFECT Repeated intake consumes clinical time, weakens continuity of care, reduces cross-service opportunities and increases the risk associated with inconsistent handling of personal information.
ESTIMATED LEAKAGE ≈ £25,000 / year Planning estimate from duplicated intakes and missed cross-sell opportunities in the supplied audit model; some GDPR and churn exposure is uncoded.	CONTROL TO INTRODUCE Adopt one GDPR-compliant practice management system as the patient single source of truth and connect it to scheduling.
03 Marketing is fragmented	LOW
OBSERVATION Each location manages its own Instagram presence; one uses a different logo. There is no shared content calendar or reliable attribution from post to booking, leaving the freelancer as a single point of failure.	DOWNSTREAM EFFECT Inconsistent presentation can weaken trust, while missing channel data makes marketing spend harder to optimise. Dependency on one person also makes continuity fragile.
ESTIMATED LEAKAGE ≈ £13,000 / year Planning estimate in the supplied audit model: lost conversions, freelancer time spent on duplicated activity and untracked/misaligned spend.	CONTROL TO INTRODUCE Use one social scheduling system, a shared brand library, AI-assisted content workflows and UTM tracking for booking attribution.

Note: the figures above are directional business estimates based on the supplied audit assumptions, not audited accounts.

Visibility and coordination are the other half of the problem

04 Cross-clinic coordination breaks down	LOW
OBSERVATION The five locations operate as separate units. One clinic may be overloaded while another has spare appointments, yet the wider team does not have a shared view of capacity.	DOWNSTREAM EFFECT Patients can be referred outside Brightpath while internal capacity remains unused. That reduces group throughput and leaves some staff under-utilised.
ESTIMATED LEAKAGE ≈ £11,000–£14,000 / year Planning estimate based on avoidable external referrals: roughly six per month × £150 average value × 12 months = £10,800, before considering multi-session courses.	CONTROL TO INTRODUCE Create one cross-clinic collaboration space, a daily capacity view and simple rules for routing patients to available Brightpath locations.

05 Management information is missing	MEDIUM
OBSERVATION There is no live dashboard. The founder cannot easily see group-wide bookings, revenue timing, retention or the most-booked service; accounting information arrives around three weeks late.	DOWNSTREAM EFFECT Pricing, staffing and expansion decisions are therefore made with delayed or incomplete evidence. Problems such as rising churn can remain hidden until the financial impact is larger.
ESTIMATED LEAKAGE ≈ £20,000 / year Planning assumption: 1% revenue drift on £2M from service-mix inefficiency and undetected churn; the risk of an uninformed expansion decision is not priced.	CONTROL TO INTRODUCE Build a lightweight dashboard from the PMS data covering bookings, no-show rate, revenue by clinic, retention, NPS and staff utilisation.

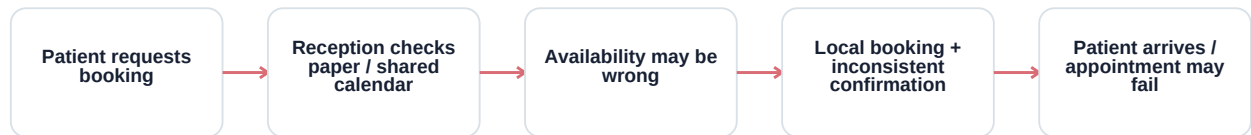
≈ £112k ESTIMATED ANNUAL OPERATIONAL LEAKAGE	≈ 5.6% OF ~£2M TURNOVER	5 CONNECTED ROOT-CAUSE GAPS
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The combined estimate should be read as a credible planning baseline rather than a precise financial statement. The case material itself notes that the objective is to establish the cost of inaction and prioritise intervention.

Fix the workflow before adding more locations

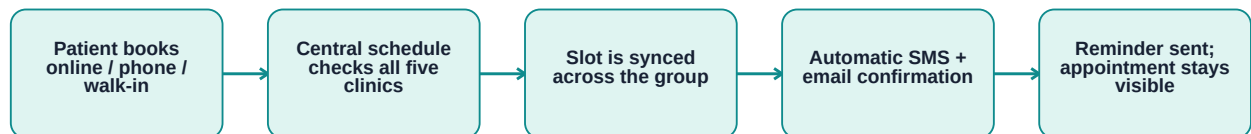
Scheduling is the clearest starting point because it is both financially material and operationally central. The future-state flow below removes the two obvious failure points: unreliable availability checks and inconsistent confirmations.

CURRENT STATE / WHERE THE PROCESS BREAKS



Failure points: unreliable availability • uneven confirmations • avoidable no-shows/rebookings

TARGET STATE / A CONTROLLED, SHARED FLOW



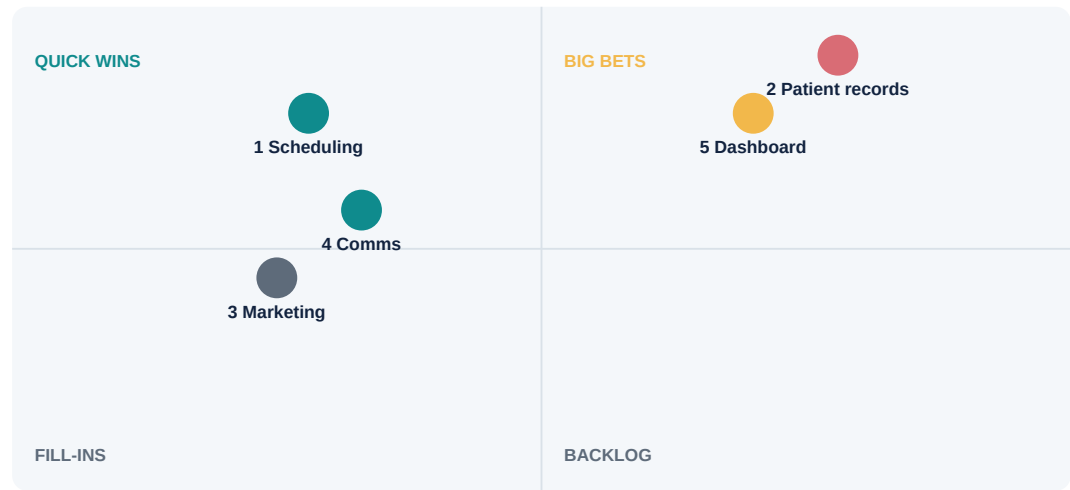
Control points: one availability record • automated confirmation • fewer preventable booking losses

PRIORITISATION

Sequence work by impact and implementation effort

The matrix is a decision aid, not a substitute for delivery planning. Scheduling is the first move; patient records are strategically important but require more effort; the remaining initiatives can follow once the core data and workflow layer is in place.

IMPACT →



EFFORT →

A practical six-month sequence

The recommended approach is deliberately staged. Start with the bottleneck that staff feel every day, establish the data layer, then use that foundation to make marketing and cross-site capacity measurable.

PHASE 1	MONTH 1
Stabilise bookings	OUTCOME ≈ £65k annual value targeted from scheduling + patient-record related improvements in the supplied audit model.
Centralise scheduling across all five clinics and start the practice-management-system rollout. Automate confirmations and reminders. This directly addresses the most visible operational pain and begins solving the cross-clinic capacity problem.	

PHASE 2	MONTH 2
Make performance visible	OUTCOME Expansion decisions move from delayed reporting toward evidence-based management.
Introduce a lightweight KPI dashboard connected to the patient-management data. Establish a weekly founder review covering bookings, no-shows, revenue by clinic, retention, NPS and staff utilisation.	

PHASE 3	MONTHS 3–6
Scale the operating model	OUTCOME Creates the operating discipline needed for a sixth and seventh location.
Centralise marketing activity, formalise cross-clinic communication and document repeatable SOPs. Use channel attribution and capacity data to improve conversion and routing.	

≈ £112,000 annual leakage identified in the audit model; ≈ £12,000 year-one software/setup cost.	Majority of the recoverable value targeted within the first quarter, with a planning payback of under three months.	Do not approve expansion until scheduling, patient records and live KPIs are operating consistently across the group.

FINAL ASSESSMENT

Brightpath does not need to reinvent its clinical proposition. It needs an operating system that matches its scale. The most defensible path is to make scheduling reliable, establish a single patient-data backbone, expose performance through a small KPI set, and then standardise marketing and cross-clinic coordination. That sequence protects service quality while making future expansion more manageable.

Prepared by Marvis Utomi • Business Operations Consultant • 21 August 2026